

- **Velvety, hyperpigmented plaques in skin folds** (axilla, neck): **Acanthosis Nigricans**. Associated with insulin resistance/diabetes and GI cancers.
- **PCT: fragile blistering photosensitivity** on dorsal hands. Assoc HCV, alcohol, hemochromatosis. Dx: urine/plasma porphyrins. Tx: avoid triggers + phlebotomy (or low-dose hydroxychloroquine).
- **Pyoderma gangrenosum**: painful ulcer with purulent base and violaceous undermined border; assoc IBD/RA. Tx: steroids.

TRAP: Pyoderma gangrenosum: do NOT debride (pathergy worsens).

- **Tender, red nodules on anterior shins**: **Erythema Nodosum**. Associated with strep, sarcoidosis, IBD, OCPs, TB.

Eczemas, Urticaria, and Drug Allergy

- **Atopic dermatitis**: pruritic, dry, scaly flexural rash. Tx: emollients daily; topical steroids for flares.
- **Urticaria (Hives)**: Pruritic, transient wheals. Treat with H1 antihistamines. If associated with anaphylaxis, give **Epinephrine FIRST**.
- **Urticarial vasculitis**: painful/burning wheals lasting >24 h that leave bruising/hyperpigmentation. Confirm with skin biopsy; assess for systemic disease (e.g., SLE, hypocomplementemia).
- **SJS/TEN**: painful **dusky bullae** with mucosal erosions after a new drug (allopurinol, sulfa, anticonvulsants). Stop the offending drug immediately; supportive care in a burn unit.

Acne and Rosacea

- **Acne Vulgaris**: Comedones (topical retinoids) -> Inflammatory papules (add topical benzoyl peroxide/antibiotics) -> Cystic (oral antibiotics -> isotretinoin).
- **Rosacea**: facial erythema/telangiectasias with papules; no comedones. Triggers: alcohol, spicy foods, heat. Tx: topical metronidazole; avoid triggers.

Contact Dermatitis

- **Allergic**: Type IV hypersensitivity (Poison Ivy, Nickel). Pruritic vesicles/bullae in **linear streaks**.
- **Irritant**: Direct chemical damage (soap, water). Burning/stinging.

Pemphigus Vulgaris vs. Bullous Pemphigoid

Key Differentiators:

- **Pemphigus vulgaris**: **Flaccid bullae** + painful **oral erosions**, **(+) Nikolsky**. Biopsy: intraepidermal cleavage (anti-desmoglein). **Mechanism of Nikolsky sign**: Intraepidermal cleavage allows epidermis to shear with lateral pressure.